



Advanced Case Management Manual for Demand Phase

Operational Protocols

Welcome to the Step-by-Step Trainee Guides. This comprehensive resource is designed to walk you through identifying, analyzing, and resolving the primary conflict areas encountered during the demand stage of a personal injury case. Follow each structured step closely to preserve claim integrity and ensure optimal outcome valuation.

Operational Note: This document contains standard operational protocols for the demand stage. *Procedures may vary depending on the rules of the firm.*

1. Protocol for Requesting Medical Record & Billing Amendments

When an audit reveals clerical or clinical errors (such as mismatched bills at Metro), execute this protocol:

1. **Isolate the Defect:** Highlight the exact line item or incorrect date of service on the document copy.
2. **Draft a Formal Request Letter:** Prepare a written request addressed specifically to the facility's Medical Records Custodian or Billing Supervisor. Phone requests are not legally binding.
3. **Include Mandatory Data:** Patient full name, DOB, account number, specific date of service being disputed, and a clear statement of fact explaining why the data is incorrect.
4. **Place a File Hold:** Mark the file as "HOLD - Awaiting Amended Billing/Records" so the demand package is not prematurely sent out with errors.

2. Auditing Strategy: Missing Records, Care Gaps, and Billing Discrepancies

Apply this cross-reference method to catch hidden gaps before insurance adjusters do:

Audit Element	What to Look For	Corrective Action Required
Treatment Gaps	Doctor notes state "return in 2 weeks" but the next visit is 45 days later (a 31-day care gap).	Contact the client to obtain an objective statement explaining the delay (e.g., illness, lack of transport) and log it.
Missing Sub-Bills	An ER visit note mentions a CT scan or an X-ray was read, but the facility ledger has no line item charge.	Issue immediate requests to the separate, third-party Radiologist Group or Physician Association.
Referrals	A doctor prescribes an MRI or diagnostic scan, but no results or reports are located in the file.	Check with the client if the scan occurred, then immediately track down the scanning provider to secure the report.



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3. Identifying Subrogation Liens and Statutory Liabilities

1. **Analyze Intake Insurance:** Establish what health coverage the client had at the time of the collision.
2. **Identify Government Claims:** If the client has Medicare or Medicaid, notify the recovery portals (MSPRP) immediately to secure a *Conditional Payment Letter*.
3. **Screen for ERISA Plans:** Private health coverage through large employers are usually self-funded ERISA plans with aggressive recovery rights. Formally request their Plan Rules and benefit statements.
4. **Map Provider Liens:** Ensure any signed contractual medical liens with local chiropractors or physicians are indexed for payout calculation.

4. Advanced Cross-Referencing of Case Data

Prior Injury History vs. Recent Claim: If a baseline check shows a historical injury to the same body part, compare the historical MRI to the new post-accident MRI. Frame the demand around an *objective structural aggravation* (e.g., a 2mm bulge that is now a 5mm herniation with impingement) rather than a simple soft-tissue injury.

Property Damage (PD) vs. Treatment Intensity: In low-impact files (under \$500 repair costs), adjusters flag cases as "MIST" claims. Mitigate this by inspecting vehicle interior photos for structural clues (e.g., a bent seat frame, cracked steering wheel) and documenting occupant positioning (e.g., head turned at impact, introducing severe rotational force).

Overcoming the MIST Defense

Insurance companies routinely use automated software to flag claims with vehicle property damage estimates under \$500 as MIST (Minimum Impact Soft Tissue) files. Adjusters argue that a low-velocity impact cannot generate sufficient kinetic energy to cause severe spinal pathology. To aggressively counter this defense, you must compile internal vehicular and biomechanical evidence.

Step-by-Step Guide to Overcoming the MIST Defense

1. Audit the Interior Cabin for Structural Damage

Low external bumper damage frequently masks catastrophic energy absorption inside the vehicle. Order a deep review of all property damage photos, salvage reports, and repair supplements. Search for and document the following:

- **Bent Seat Frames or Tracks:** This is evidence that the forces overrode the energy-absorbing bumper mechanism and sheared or bent the driver/passenger seat steel.
- **Steering Wheel and Dashboard Defects:** Look for cracked steering wheel plastics, knee-strike markings on the lower dash column, or deployed seatbelt pretensioner locks.
- **Bumper Reinforcement Cracks:** Check repair supplements for hidden damage behind the plastic fascia, such as a crushed foam absorber or a bent rear impact bar that would not appear in surface-level photos.

2. Document and Reconstruct Occupant Position Mechanics

The force vector combined with the occupant's precise body positioning dictates injury severity far more than external sheet metal damage. Detail the following inside the demand narrative:

- **Asymmetric or Rotated Posture:** Determine if the client's head was turned fully to the left or right (e.g., checking a side-view mirror or talking to a passenger) at the moment of impact. Rotational cervical extension introduces complex twisting forces into the spinal ligaments and facet joints, which severely heightens structural vulnerability even at speeds as low as 5 mph.
- **Bracing and Foot Vector Placement:** Note if the client was forcefully pressing down on the brake pedal during a rear-end collision. This rigid bracing bypasses normal joint damping, transferring kinetic force directly up through the ankle, knee, hip, and lumbar spine, resulting in traumatic joint shear.



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5. Unresolved and Completely Unhandled Property Damage Claims on Demand

When the property damage claim was completely unhandled during the pre-suit phase (meaning no repairs have been made, no total loss evaluation completed, and the car damage remains untouched), it must be integrated into the global Bodily Injury (BI) demand package to protect the client's comprehensive property interest.

- **Obtain an Independent Repair Estimate:** Deploy an independent appraiser to secure an itemized repair blueprint.
- **Total Loss NADA/KBB Valuation:** Establish a blue-book retail value anchor if structural frame damage is suspected.
- **Compile Photographic Evidence:** Secure a full 360-degree point-of-impact set, focusing on undercarriage and crumple zones.
- **The Release Trap Safeguard:** Verify that any partial property settlement states: ***"Partial Settlement for Property Damage Line Item Only – Bodily Injury Action Explicitly Reserved."***

6. Managing Active Bodily Injury (BI) Liability Disputes During the Demand Stage

Submitting an administrative demand package while the adverse insurer explicitly disputes liability requires a high-level preemptive evidentiary strategy. CM must transform the administrative narrative into an undeniable liability packet.

- **Deconstruct the Police Report (PR):** Explicitly quote specific state vehicle code violations within the introductory sections of the demand.
- **Affidavit and Witness Statements:** Formally transcribe verbal witness testimony and integrate them as indexed exhibits to lock down physical crash facts.
- **Black Box (EDR) and Video Canvassing:** Anchor spatial vehicle positions using surrounding CCTV, commercial security cams, or dashboard recordings.
- **Preemptive Comparative Response Strategy:** Map remaining high-value objective diagnostic records to establish that defendant exposure remains absolute despite minor shared-fault arguments.
- **The Litigation Escalation Threshold:** Route the file directly to the firm's litigation division for immediate formal filing if a total liability denial persists.

7. Managing Client and Ethical Conflicts

A. Double Representation Conflict (Common Representation)

When representing multiple clients from the same accident, a conflict exists where the settlement of one claim may materially limit the settlement potential of another claim.

Demand Stage Risks

- The settlement of one claim may hurt the settlement potential of another (e.g., little money remaining).
- One party's refusal to settle may prevent the other from settling, especially when the defendant will only settle all claims at once.
- Knowledge of privileged information concerning one client can potentially be used against the other claim.

Mandatory Protocol

Obtain written informed consent from all affected clients after fully explaining the risks and advantages of common representation.

B. Other Key Conflicts Arising in the Demand Stage

Client-Related Conflicts (Credibility Risks)

- **Inconsistent Statements:** Cross-examine client statements recorded across intake, treatment notes, and current demand preparation.
- **Prior Injury Disclosures:** Confirm that all historical injuries and prior accidents were fully disclosed.
- **Social Media Risks:** Scan for public photos or videos that directly contradict injury claims.

Treatment & Financial Conflicts

- **Treatment Progression:** Track the logical progression from conservative care to specialist care to confirm appropriate treatment escalation.
- **Red Flags:** Watch for excessive treatment or minimal treatment despite severe client complaints.
- **Net Settlement Concerns:** Run calculations to determine net settlement concerns and ensure the client is adequately protected from financial obligations.

7. Mandatory "Demand-Ready" Final Audit Checklist



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Checklist Item	Verifiable Audit Standard Required
1. MMI Status	Client has been formally discharged or has a future care plan with explicit cost projections.
2. Ledger Balance	Bottom-line outstanding balances on provider ledgers match our internal Specials Summary to the penny.
3. Lien Payoffs	All statutory subrogation lienholders have issued an updated statement within the last 30 days.
4. Wage Verification	The file contains an official Lost Wage Form signed by HR, accompanied by matching paystubs.
5. Organization	All medical charts, bills, and diagnostics are sequentially indexed into numbered PDF Exhibits.